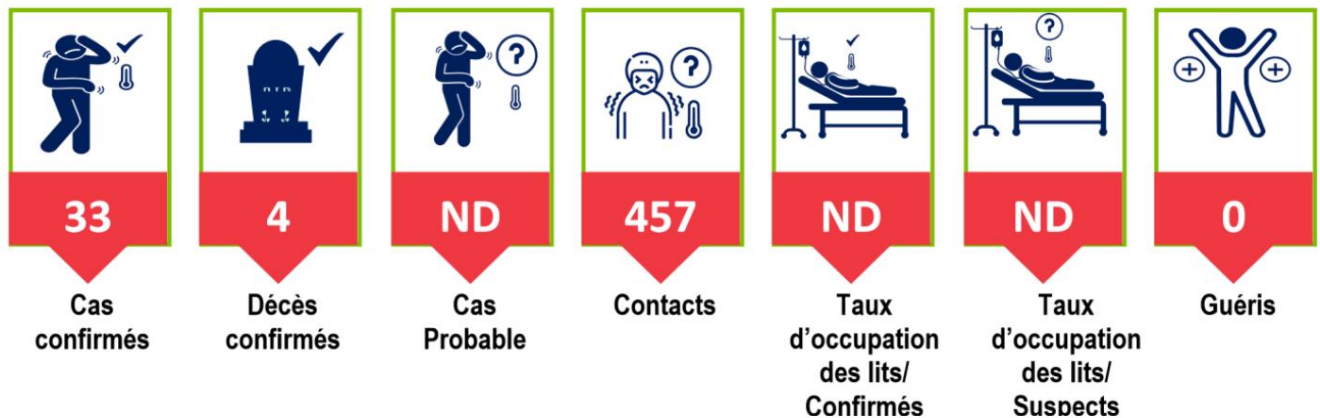




Ebola Virus Disease Situation Report/ 17th Epidemic - DRC

SitRep MVE No. 004/2026

Affected provinces	ITURI, NORTH KIVU
Affected Health Zones	ITURI : Mongbwalu, Rwampara, Bunia, Nyankunde, NORTH KIVU : Goma, Katwa and Butembo
Reporting date	May 19, 2026
Publication date	May 20, 2026



Provinces touched	Confirmed cases	Death confirmed	Probable cases	Suspected cases	Healed by the day
Ituri	30	4	ND	131	0
North Kivu	3	0	ND	ND	0
Total	33	4	ND	0	0



Descente du ministre de la santé publique, hygiène et prévoyance sociale, le DG de l'INSP, le coordonnateur du COUSP et les experts COUSP-OMS pour renforcer l'équipe provinciale de l'Ituri à répondre à l'épidémie de la maladie à virus Ebola (Bundibugyo).

I. CONTEXTE

The Ituri province is located in the northeast of the Democratic Republic of Congo and shares a long border with Uganda and South Sudan. For more than two decades, it has faced a chronic humanitarian crisis linked to armed conflict and recurring population displacements. It has an estimated population of over 8 million, including more than one million internally displaced persons.

In September 2018, this province was affected by an Ebola virus disease epidemic that was raging in the neighbouring province of North Kivu.

The Mongbwalu health zone is one of the 36 health zones in Ituri province. Considered the starting point of the epidemic, it is located in the Djugu territory, where several armed groups operate. It is situated 70 km from Bunia, the capital of Ituri province, and experiences frequent population movements towards Uganda. This area is characterized by numerous construction projects that attract people from other territories within the province and from neighboring provinces.

I. **History of the epidemic**

On May 5, 2026, social media reported a surge in deaths of unknown cause, with nearly 50 deaths already recorded in the Mongbwalu health zone. Following confirmation of this alert, the Chief Medical Officer of the Mongbwalu health zone shared the preliminary investigation report on May 9, 2026. On May 11, 2026, provincial health authorities held an emergency meeting and decided to deploy rapid response teams to Mongbwalu and Rwampara, which had reported suspected deaths from viral hemorrhagic fever among patients hospitalized at the Rwampara Evangelical Medical Center, who had been transferred from Mongbwalu.

Investigations conducted on May 12, 2026, simultaneously in the Mongbwalu and Rwampara health zones by the Rapid Intervention Teams of the Ituri Provincial Health Division (DPS), revealed that most suspected patients or deaths presented with fever, headache, vomiting, and severe physical asthenia. Deaths occurred 2 to 3 days after the onset of illness, with symptoms including hematemesis, epistaxis, or blackish vomit. During the investigations in the Rwampara health zone, 20 samples were collected from suspected cases or deaths. The majority of samples were taken from the Mongbwalu health zone. Preliminary tests conducted at the Bunia Laboratory were all negative for Ebola Zaire, dengue fever, rotavirus, cholera, malaria, Yersinia pestis, measles, and COVID-19. These samples were sent to the National Institute of Biomedical Research (INRB) in Kinshasa for further testing.

*As of May 14, 2026, **13 of these 20 samples were tested, and 8 were found to be positive** for non-Zairean Ebola virus disease at the INRB Kinshasa. On May 15, 2026, sequencing of the positive samples identified the Bundibugyo strain, allowing the Minister of Public Health, Hygiene and Social Welfare (MSHPS) to officially declare the 17th Ebola virus disease outbreak.*

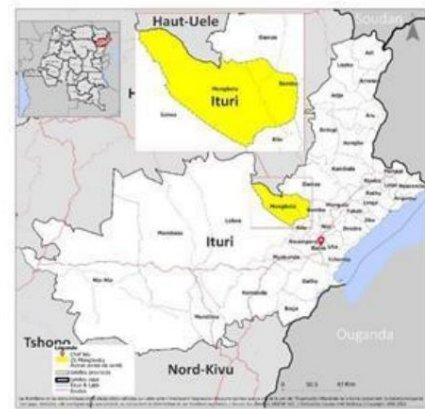
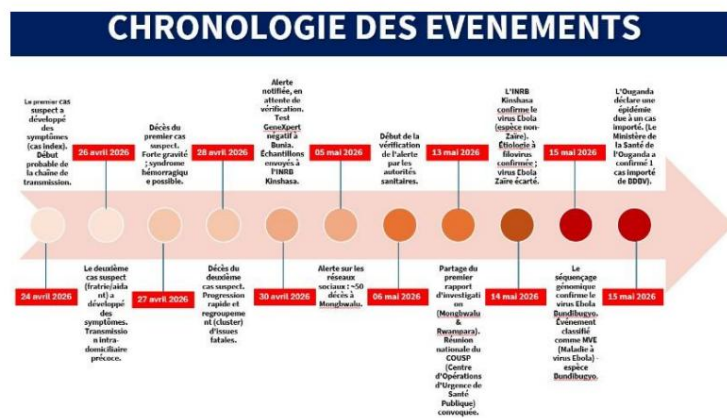


Figure 1. Location of the Mongbwalu health zone in Ituri province.

II. POINTS SAILLANTS

- 20 New confirmed cases as of May 18, 2026, including 18 in Ituri province and 2 in the province of North Kivu.
- As of May 18, 2026, cumulative total of 33 confirmed cases including 30 confirmed cases in Ituri and 3 in North Kivu - Two provinces affected including Ituri (4 ZS: Bunia, Nyankunde, Mongbwalu and Rwampara) and North Kivu (3 ZS: Katwa, Goma and Butembo).
- The farmers at the TSERE farm in the Rwampara health zone were made aware of the need to avoid consuming the meat of the 5 pigs that died from an unidentified disease, and two of which showed signs of hemorrhage. As a result, the 5 dead pigs were disposed of.

III. MISE A JOUR DE LA SITUATION

Table I : Distribution of confirmed and probable cases and deaths of Ebola virus disease in the health zones of the Ituri and North Kivu Provincial Health Directorates as of May 18, 2026*

Provinces	Health zones	Number of suspected cases	Number of suspicious	Number of cases Confirmed	Number of contacts
Ituri	Mongbwalu	302	deaths : 74	1	161
	Rwampara	136	38	19	120
	Bunia	61	18	6	127
	Nyankunde	14	1	4	133
North Kivu	Goma	1	0	1	ND
	Katwa	1	0	1	ND
	Butembo	1	0	1	ND
Total		516	131	33	541

*This data is subject to change after harmonization of data at different levels of the health pyramid with the incident management system.

Table II : Daily alert status in the health zones of the Ituri DPS as of May 18, 2026

Indicators on the alerts	Bunia	Rwampara	Nyankunde	Mongbwalu	Nizi	Lita	Bambu	Fataki	Total
Alerts postponed (May 17th)	3	7	0	0	0	0	ND	ND	10
New alerts (18/05)	13	23	4	11	1	2	ND	ND	54
Total alerts from day (18/05)	16	30	4	11	1	2	ND	ND	64
Live alerts	10	20	3	9	1	2	ND	ND	45
Deceased alerts	6	10	1	2	0	0	ND	ND	19
Alerts investigated	16	22	4	9	1	2	ND	ND	54
No alerts investigated	0	0	0	9	0	0	ND	ND	9
Alerts confirmed	16	21	4	9	1	2	ND	ND	53
Alerts invalidated	0	1	0	2	0	0	ND	ND	3
Suspected cases sampled	10	21	4	2	0	0	ND	ND	37
Isolated suspected cases	9	46	3	9	0	2	ND	ND	69
Suspected cases not isolated	1	0	0	0	1	0	ND	ND	2

III. ACTIONS DE REPONSE

COORDINATION

- Holding the coordination meeting with the sections. • Participation in strategic meetings at the governorate and central level. • Holding the coordination meeting with the sections.
- Participation in strategic meetings at the governorate and provincial levels • Sharing priority activities and budgets with the central level • Establishment and revitalization of intervention teams • Identifying potential sites for CTE installations

SURVEILLANCE EPIDEMIOLOGIQUE

- Briefing of surveillance teams on alerts, investigations, active search, listing and tracking contacts;
- Briefing the PoE/PoC team on mapping their sites;
- Organization of the surveillance pillar (PF by ZS, roles and responsibilities);
- Deployment of surveillance teams in 3 health zones experiencing an epidemic: Bunia, Rwampara and Nyankunde

LABORATOIRE

Results as of May 18, 2026 (RADIONE)

- Samples received: 27 (New suspected cases) • Samples analyzed: 27 • Samples analyzed: 0
- Confirmed cases today: 18 (Rwampara: 6, Bunia: 8, Mongbwalu: 1, under verification: 3), 40 samples awaiting analysis at the Ituri Laboratory and 54 samples from Mongbwalu awaiting shipment to the Ituri lab. • Cumulative confirmed cases: 33 • Ituri Laboratory: 23

INRB Kinshasa: 7

o Goma Laboratory: 1

Butembo Laboratory: 2

RISK COMMUNICATION AND COMMUNITY ENGAGEMENT (RCCE)

- Awareness-raising of 56 tuberculosis patients, including 7 women, in the Mudzi-Maria health area; • Awareness-raising of 26 CAC members, including 12 women, in the Kasegwa health area to activate VADs in all avenues and villages;
- Briefing of the 23 PRESICODESA (local health centers) in the Bunia health zone on the CREC (Regional Community Health Center); • Capacity building by the central level on writing the daily report for the team from the pillar secretariat.

PREVENTION ET CONTRÔLE DE L'INFECTION (PCI/WASH)

ZS Bunia

- 3 EDS (Emergency Diagnostic Sites) completed and decontamination: Bunia General Hospital, Nyankunde Primary School, Health Center. The merits include: • Initial briefing of supervisor and contaminant teams, • Site assessment for triage implementation at the DPS (Department of Health Promotion) office. Ituri;
- Briefing of technicians on the maintenance and disinfection of meeting spaces DPS,
- Evaluation of fire protection devices at the DPS and LPSP level (9 fire extinguishers for LPSP and 12 for DPS) all to be maintained.

ZS Rwampara

- Start of the assessment at the Nyankunde Medical Center; 18 suspected cases present but without individually, including 8 healthcare providers,
- Briefing of 68 service providers on Ebola Virus Disease (EVD) • Clinical evaluation of Dieu Guéri: scorecard of 28%
- Disinfection of several rooms in the internal medicine ward of the Medical Center (CME), Tumaini Clinic, and cleaning. • Identification of a site for patient isolation

ZS Mongbwalu

- Briefing of 5 service providers from HGR Mongbwalu, • Decontamination of isolation unit, • Provision of Wash kits in 4 CS by ACF.

LOGISTIQUE

- Identification of vehicle fleet: • Ambulances: 4 (Bunia General Hospital, Mongbwalu General Hospital, PNC and UMIR); • Programs and Coordination: 3 (CPLT, MTN, PEV) • WHO: 5 vehicles for the surveillance pillar; • UNICEF: 6 rented vehicles, including 1 for SBC;

SECURITE

- The security situation in the city of Bunia (Bunia and Rwampara health zones) is relatively calm

IV. DEFIS

- Difficulty in team mobility for investigations at the DPS level and ZS
- Difficulty in shipping samples from Mongbwalu to the provincial laboratory in Bunia
- Insufficient sampling kits • Insufficient protective kits and monitoring tools

V. RECOMMANDATIONS

- Reinforce intervention teams in the affected health zones, • Make transportation (mobility) available to field teams, • Fill the gap in sampling kits, • Continue compiling contact lists and begin contact tracing in all areas

ZS,

- Develop a map of population mobility in health zones affected,
- Strengthen epidemiological and community-based surveillance, • Train service providers in epidemiological surveillance, community-based surveillance, and laboratory work

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